

MEDCORE HOSPITALS

ICU Operations Guidelines

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1. Purpose and scope

Provides operational guidance for ICU admission, bed allocation, handoffs, movement, support coordination and review.

Department interfaces

Clinical teams retain responsibility for clinical decisions and monitoring. Clinical Operations coordinates destination, capacity, movement, support functions and operational exceptions.

Use of this document

This document is an operational reference. It is designed to help staff understand the normal sequence, identify the responsible function, maintain a visible status and close or hand over work correctly. It does not replace clinical judgement, professional standards, emergency response requirements or other approved hospital policies. Where a more specific departmental or clinical procedure applies, that procedure remains the governing reference.

2. Operating principles

The workflow should begin with the correct patient and encounter, continue through an identifiable owner, and finish only when the operational state has been reconciled. Staff should avoid creating parallel informal records when the approved system or form is available. When work cannot proceed, the dependency should remain visible, the next action should be identified and the matter should be escalated through the designated route when necessary.

3. Intake and verification

The intake stage establishes the operational identity of the work and makes the first owner visible. The objective is to prevent avoidable rework later in the workflow by resolving basic status, destination and ownership questions early.

Initial request

The request should be linked to the correct patient encounter and should identify the service or pathway being requested. Staff should use the approved hospital system or form and should avoid copying the same request into multiple unofficial lists. If information is incomplete, the missing item should be stated clearly so that the responsible function can act without repeatedly returning the request for clarification.

- Confirm the patient and encounter before attaching operational information.
- Identify the requesting and receiving functions.
- Record the intended next stage where known.
- Make any dependency that prevents progress visible.
- Assign an accountable owner for open work.

Verification and reconciliation

Verification is particularly important when information has been entered by more than one department. If two records appear inconsistent, staff should use the authorized correction or reconciliation process rather than choosing the version that seems most convenient. A discrepancy should remain visible until the responsible function confirms resolution.

- Use approved identifiers and records.
- Do not create duplicate records to bypass an unresolved discrepancy.
- Record the nature of an operational discrepancy factually.
- Route identity or records issues to the authorized function.
- Confirm the corrected state before proceeding.

Operational record and handoff

The record should show what was requested, when it entered the workflow, its current state and who owns the next action. A receiving team should be able to understand the operational position without reconstructing it from old messages.

Exception and escalation

When intake cannot proceed, keep the item open and use the relevant escalation route. Do not mark the stage complete merely because the request has been received.

Completion check

Patient and encounter are correct; the request and destination are understood; open dependencies have owners; and the next operational action is visible.

3. ICU admission, bed allocation and readiness

ICU operations connect an authorized admission or transfer pathway with a prepared receiving location. Clinical decisions remain with the responsible clinical team while Operations coordinates bed and movement dependencies.

Bed allocation

ICU capacity should be coordinated through the approved bed-management process. Because availability can change, a previously discussed bed should not be treated as confirmed until the required status is established. Changes should be visible to sending and receiving functions.

- Check current capacity.
- Confirm the intended unit and bed.
- Record assignment through the approved process.
- Keep unavailable capacity visible.
- Escalate material capacity constraints.

Readiness

Preparation should confirm the operational prerequisites for receiving the patient, including destination readiness and relevant support arrangements. Specialist clinical or infection-control requirements remain with the responsible functions.

- Confirm receiving readiness.
- Identify missing support requirements.
- Assign owners.
- Recheck after material changes.
- Keep dependencies visible.

Operational record and handoff

The ICU record should connect the admission or transfer request with current bed status, destination and receiving ownership.

Exception and escalation

If the receiving location is not ready, keep the request open and escalate the capacity or preparation dependency.

Completion check

The receiving destination is confirmed and the relevant operational readiness state is reconciled.

4. Coordination and preparation

Once the initial request is verified, coordination connects the requirement with the people, location and resources needed for the next stage. Preparation should be practical and proportionate: complete what is required, identify what is missing and avoid duplicating work already documented elsewhere.

Resource and destination coordination

Where a bed, room, service, equipment item or receiving function is required, the responsible coordination team should use the approved capacity or allocation process. Availability can change, particularly during periods of high activity, so a previously discussed destination should not automatically be treated as confirmed. Changes should be reflected in the current operational status and communicated to the affected teams.

- Confirm destination or service through the approved process.
- Check current availability rather than relying on an old message.
- Identify preparation requirements.
- Record material changes to the plan.
- Keep unresolved capacity issues visible.

Readiness review

The readiness review is a coordination step rather than a substitute for professional clinical assessment. Its purpose is to identify whether the operational prerequisites for the next step have been addressed. Where an item remains pending, the record should show the item, the responsible function and the expected next action.

- Separate completed work from pending work.
- Assign an owner for each material dependency.
- Use existing hospital forms and systems.
- Notify the receiving function of significant changes.
- Recheck readiness when circumstances change.

Operational record and handoff

The operational record should provide a shared view of destination, readiness, outstanding work and ownership. This reduces the risk of one department working from an outdated status while another department has already changed the plan.

Exception and escalation

If capacity, equipment or preparation is unavailable, record the dependency and escalate according to the approved route. Avoid informal workarounds that cannot later be reconciled.

Completion check

Destination and preparation status are current; outstanding dependencies are visible; and the receiving function knows what remains to be completed.

5. Handoff and movement

Movement is a transition between accountable functions. The operational handoff should make the sending location, receiving location, timing and pending work clear while clinical teams conduct the clinical handoff under their professional procedures.

Handoff

A useful handoff does not need to reproduce every detail from the originating record. It should preserve the information needed for the next operational action. The sending function should identify what has been completed, what remains open and who is expected to act. Where acknowledgement is required, it should be captured through the approved process.

- Identify sender and receiver.
- Confirm the current location.
- State the next operational action.
- List material pending items.
- Record acknowledgement where required.
- Use approved communication channels.

Movement and arrival

A physical move should be followed by an operational reconciliation. The actual destination should agree with the recorded destination, and the receiving function should be identifiable. If the patient or task reaches a different location than originally planned, the changed state should be corrected before the workflow is closed.

- Confirm the actual destination after movement.
- Update location status promptly.
- Reconcile bed or resource status.
- Record material changes in timing or destination.
- Escalate an unexpected movement when ownership is unclear.

Operational record and handoff

The handoff record should distinguish the current state from historical states. The receiving team should not have to infer whether an old destination is still active.

Exception and escalation

If acknowledgement or arrival cannot be confirmed, keep the transition open and use the approved escalation route. Silence should not be treated as confirmation.

Completion check

The receiving location is confirmed, ownership is clear, the relevant resource state is reconciled and the handoff is complete.

6. Delays, exceptions and downtime

Operational delays are expected in a complex hospital environment. The control is to make the delay visible and actionable rather than allowing it to disappear between shifts, systems or departments.

Delay management

A useful delay record identifies what is waiting, why it is waiting at the level currently known, who owns the next action and what should happen next. Staff should avoid speculative explanations or personal attribution. If the reason is still being investigated, the record should say that rather than presenting an unverified cause as fact.

- Classify the operational dependency.
- Assign the next-action owner.
- Record the next review or action.
- Escalate material barriers.
- Update the status when circumstances change.
- Keep the original request traceable.

System downtime

When an approved downtime procedure is activated, temporary records should be controlled and traceable. Staff should use the designated downtime process rather than inventing a permanent parallel workflow. When normal systems return, the relevant team should reconcile temporary information into the approved record and resolve any duplicate or conflicting states.

- Use the approved downtime procedure.
- Keep temporary records controlled.
- Identify the reconciliation owner.
- Record material events required for later reconciliation.
- Complete reconciliation after restoration.

Operational record and handoff

Open exceptions should remain visible during shift changes. A new staff member should be able to see the current issue, owner and next step without depending on informal memory.

Exception and escalation

Escalate issues that cannot be resolved within the normal route or that create a material operational risk. Do not close an exception simply to remove it from an active list.

Completion check

Every unresolved issue has an owner and next action; temporary states are controlled; and the approved workflow reflects the current situation.

7. Communication, privacy and documentation

Communication should support continuity while remaining within the staff member's role and the hospital's privacy requirements. Operational notes should be factual, relevant and sufficient for the next action.

Communication practice

Staff should use approved communication channels and confirm that the recipient is appropriate for the information being shared. Operational communication may cover location, timing, ownership and logistics. Clinical questions should be directed to authorized clinical personnel. Where a communication changes the workflow, the corresponding operational status should also be updated.

- Use the approved channel.
- Confirm the appropriate recipient.
- Share only information necessary for the task.
- Record material operational communication where required.
- Route clinical questions to the appropriate professional.

Documentation quality

Good documentation makes the workflow easier to continue. It should distinguish completed actions from pending actions and should avoid unsupported conclusions. When a correction is needed, the approved correction method should preserve traceability rather than replacing the original history without explanation.

- Use clear factual language.
- Record material status changes.
- Keep ownership visible.
- Avoid unnecessary duplication.
- Follow the approved correction process.

Operational record and handoff

The operational record should be concise enough to use during a busy shift while still preserving the information needed to understand the current state.

Exception and escalation

If the correct recipient, channel or documentation method is uncertain, pause and use the approved escalation or supervisory route.

Completion check

Communication is complete through an approved channel, material changes are documented and the record supports the next operational action.

8. Monitoring and continuous improvement

Routine review can identify recurring workflow friction such as capacity delays, unclear ownership, repeated handoff gaps or documentation problems. Review should remain based on observable events and should produce actionable follow-up.

Operational review

Review teams can examine open items and recurring exceptions to identify where a process owner may need to intervene. The purpose is not to create additional narrative but to understand whether the documented workflow is being followed and where the workflow itself may need clarification. Observations should be separated from hypotheses about causes.

- Review recurring open items.
- Identify repeated handoff or coordination gaps.
- Check whether owners were assigned.
- Record improvement actions.
- Track completion of agreed actions.

Ownership and follow-up

An improvement item should have an owner and a follow-up state. If an issue requires investigation, the review record should make that explicit. Once the action is completed, the result should be recorded in the appropriate system or review mechanism.

- Assign the relevant process owner.
- State the action required.
- Set a follow-up point where applicable.

- Record completion.
- Escalate recurring unresolved issues.

Operational record and handoff

Review records should distinguish the observed issue, agreed action and accountable owner. This provides a useful trail without turning the SOP into a performance-reporting document.

Exception and escalation

Recurring issues should be routed to the responsible process owner rather than repeatedly recorded without follow-up.

Completion check

Open improvement items have owners and follow-up, and the current operational process remains aligned with the approved workflow.